You've Started to Wonder About Mom: Three Places to Begin

Three first moves, in order, and what can wait.

Nothing urgent is happening. Mom still lives at home and manages most things herself. You've noticed a few small changes, and you're not sure what to look at first.

Start here. Three things, in this order. If the third one comes up by itself before you've done the other two, take it.

1 Write down an ordinary week

For one ordinary week, note what Mom does on her own and where someone already gives her a hand: bathing and dressing, meals, shopping, housework, medications, the bills. Put the date at the top. These are among the everyday tasks the federal Administration for Community Living lists when it describes long-term care. [1]

WHY THIS COMES FIRST Everything else starts from how she is doing now. A dated page also gives you something to compare against later, so nobody is relying on memory.

2 Keep a list of questions, and who each one goes to

When you notice something about her health, her memory, her medications or her steadiness, write it down as a question for her doctor or her pharmacist. When an older person is concerned about falling or balance, the Administration for Community Living's advice is to discuss it with a health care provider. When the trouble is keeping track of medicines, it is to discuss it with the doctor and the pharmacist. [2]

WHY THIS COMES FIRST The week of notes is where the questions come from. Writing each one down means you don't have to work out what a change means. You only have to ask.

3 Ask Mom what she would want

Pick a quiet time and ask her, in open questions, where she would prefer to be cared for if she ever needed more help, how much staying in her own home matters to her, and who she would want involved in decisions. Write down what she says in her own words.

WHY THIS COMES FIRST Nobody can plan around her wishes until someone asks her. The first two moves give you something real to talk from.

What can wait

- Which kind of care would fit Mom, and when. That depends on her needs, and it is a question for the people who assess her.
- What care would cost, and how it would be paid for.
- Who in the family would do what in the long run.
- What your notes mean. A plain, dated week of them is what makes the later conversations good ones.

One thing that isn't for the notebook

If Mom tells you she is worried about falling, about dizziness or about her balance, the Administration for Community Living suggests she discuss it with her health care provider, who can assess it. That is a conversation for her and her provider. It isn't something to settle from a week of notes.

What this guide doesn't do

It doesn't say what a change means, whether Mom is safe at home, or what kind of care she may need, and it isn't medical, legal, financial or tax advice. It puts three first moves in order.

Ready for the next step?

You have a dated week of notes, a list of questions and Mom's wishes in her own words. The care interview takes it from there: it walks you through her situation area by area and turns your answers into a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

The everyday tasks to note, and the advice to take questions about medicines, falling and balance to a doctor or pharmacist, follow the Administration for Community Living. The three moves and their order are ours.

1. Administration for Community Living (LongTermCare.gov), What is Long-Term Care?
2. Administration for Community Living (LongTermCare.gov), Avoiding a Fall

From the Medicaid Planning Roadmap · Richard Ladson, Certified Medicaid Planner. Draft for review, generated 2026-10-04.